

Legal Causation (2)

Topics in legal causation

1. Remoteness of damage
2. New intervening acts
3. Supervening events
4. 'The scope of duty' principle
5. Coincidental harms



3. Supervening Events

Baker v Willoughby [1970] AC 467



Facts:

- ☐ D negligently injured C's leg, forcing C to take another job.
- ☐ While working in the new job, but before the case came to trial, burglars shot C in the same leg.
- ☐ C's leg then had to be amputated.

Held:

- ☐ D still liable.

Jobling v Associated Dairies Ltd [1982] AC 794



Facts:

- ❑ C injured his back at work, reducing his earning capacity.
- ❑ Before the trial against his employer, C contracted an unrelated back disease which rendered him completely incapable of any work.

Held:

- ❑ D not liable after the new disease hit.

Summary: supervening events

We can reconcile *Baker* and *Jobling* by adopting the rule that:

- ❑ later torts which negate an earlier tort's damage, do not negate D1's liability; but,
- ❑ later natural events which negate an earlier tort's damage, do negate D1's liability.

BUT...maybe *Baker* is just wrong?

- ❑ “I can formulate no convincing juristic or logical principles supportive of the decision of this House in *Baker v Willoughby*, and none were there propounded.” (Lord Edmund-Davies in *Jobling*)



4. The Scope of Duty Principle |

The SAAMCO rule

South Australia Asset Management Corp v York Montague Ltd [1997] AC 191 ('SAAMCO')

“Normally the law limits liability to those consequences [of wrongful conduct] which are attributable to that which made the act wrongful.”

(Lord Hoffmann, [213])

The *SAAMCO* rule



“A mountaineer about to undertake a difficult climb is concerned about the fitness of his knee. He goes to a doctor who negligently makes a superficial examination and pronounces the knee fit. The climber goes on the expedition, which he would not have undertaken if the doctor had told him the true state of his knee. He suffers an injury which is an entirely foreseeable consequence of mountaineering but has nothing to do with his knee. ... [T]he doctor is not liable. The injury has not been caused by the doctor's bad advice because it would have occurred even if the advice had been correct.”
(Lord Hoffmann, [213])

The *SAAMCO* rule



Thus:

- ❑ the doctor would be liable if mountaineer falls while climbing because his knee buckled, but
- ❑ would not be liable if the mountaineer gets hit by a falling rock,

even though he would never have gone on the expedition had the doctor not been negligent.

The SAAMCO rule

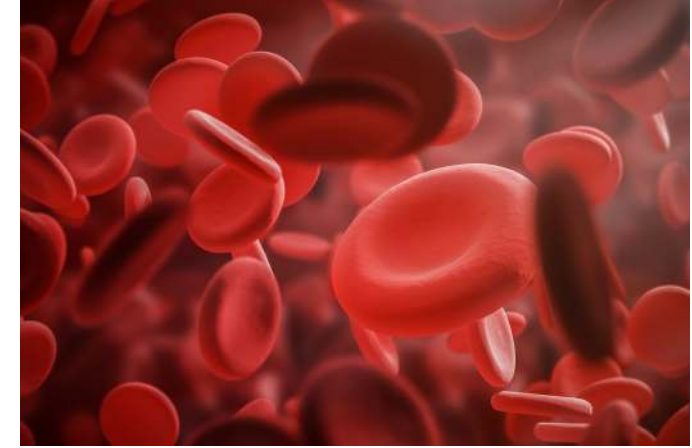
“the focus should be on identifying the purpose to be served by the duty of care assumed by the defendant”

(*Manchester Building Society v Grant Thornton UK LLP* [2021] UKSC 20)

In the mountaineer example, the purpose of the doctor’s advice was to certify that C’s knee was fit for purpose. So:

- ❑ D is liable for any losses which result from C’s knee not being fit for purpose (inside the scope of D’s duty),
- ❑ but D is not liable for other losses, even though his negligence is the cause (outside the scope of D’s duty).

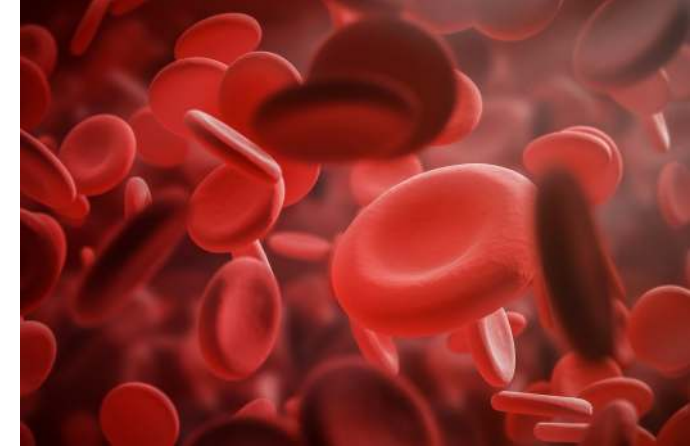
Khan v Meadows [2021] UKSC 21



Facts:

- ❑ C asked D if she carried the gene for haemophilia so that she could avoid having a haemophilic child.
- ❑ D's negligence left her with the impression she did not and, in due course, her son was born.
- ❑ Her son suffered from severe haemophilia AND autism.
- ❑ C would not have had the baby if she knew she carried the haemophilia gene.

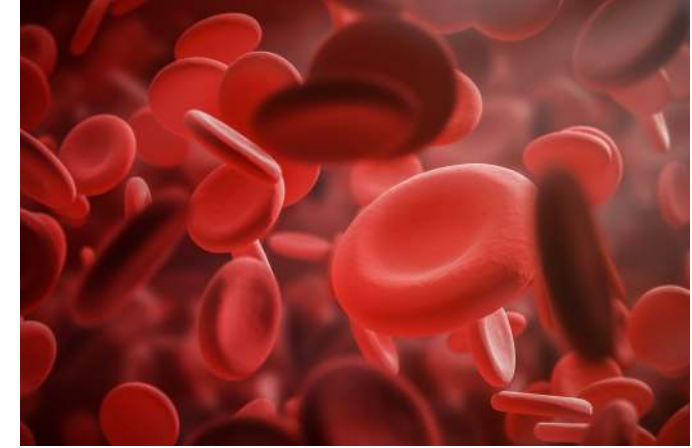
Khan v Meadows [2021] UKSC 21



Held:

- ❑ D only liable for the haemophilia, not the autism.
- ❑ “the scope of duty question is answered by addressing the purpose for which Ms Meadows obtained the service of the general medical practitioners. She approached the general practice surgery for a specific purpose. She wished to know if she was a carrier of the haemophilia gene. ... Dr Khan owed her a duty to take reasonable care to give accurate information or advice when advising her whether or not she was a carrier of that gene. ... The important point is that the service was concerned with a specific risk, that is the risk of giving birth to a child with haemophilia.” (Lords Hodge & Sales, [67])

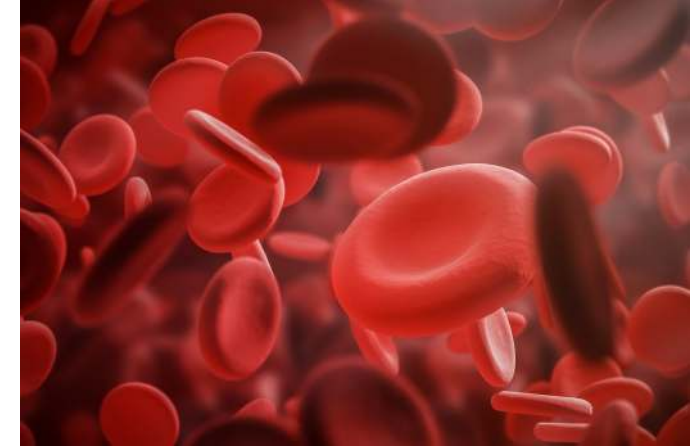
Khan v Meadows [2021] UKSC 21



Lord Burrows

“The purpose of the advice or information is of central importance. The claimant had approached the general practice surgery, as the defendant knew or ought to have known, for the specific purpose of ascertaining whether or not she was a carrier of haemophilia and hence what the impact of that would be if she were to become pregnant. The purpose of the advice or information was not to ascertain the general risks of pregnancy, including the risk of autism.” [77(i)]

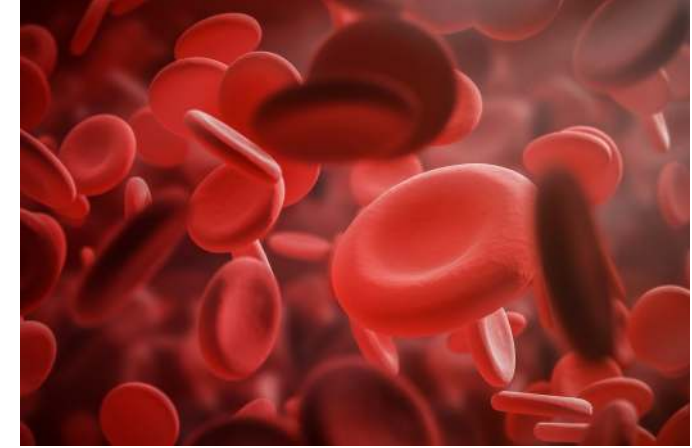
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Lord Burrows

“In the light of that purpose, it was fair and reasonable that the risk of the child being born with haemophilia should be allocated to the doctor; but that the risk of the child being born with autism should be allocated to the mother.” [77(ii)]

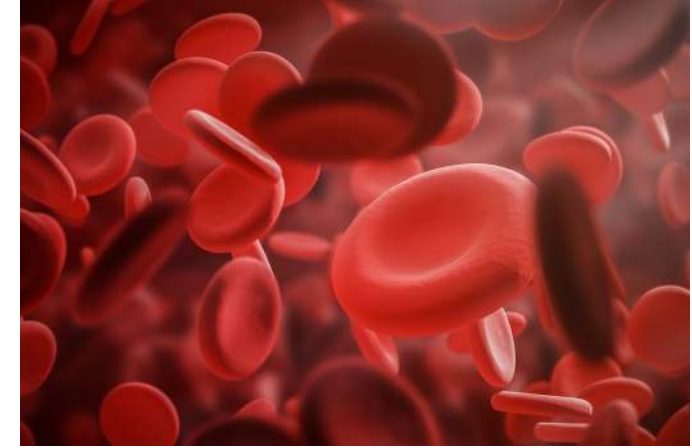
Khan v Meadows [2021] UKSC 21



Lord Burrows

“If we ask the question, would the claimant have suffered the same loss had the information/advice been true, the answer is “yes” as regards the autism losses ... but “no” as regards the haemophiliac losses ... This is because had the information/advice that the claimant was not a carrier of haemophilia been correct, the claimant would still have given birth to an autistic child but would not have given birth to a child with haemophilia.” [77(iii)]

Khan v Meadows [2021] UKSC 21



Lord Leggatt

“The appeal turns on whether or not there was also a causal connection between the fact that the claimant was carrying a gene for haemophilia and the autism from which [the child] suffers. That question is answered conclusively by the parties’ agreement that the autism was not caused by his haemophilia nor made more likely by it. It follows that the costs associated with his autism are not within the scope of the defendant’s duty of care.” [93]

Is this different from normal remoteness?

- ❑ “if a particular kind of harm is not foreseeable then the risk of that harm occurring cannot be one of the risks that made the defendant’s conduct negligent”.
- ❑ “[T]he underlying idea is a simple one. Unfortunately, however, that simplicity has come to be obscured ...: first, because of the tendency to associate the principle with a particular case which happened to concern professional advice [*SAAMCO*] ...; and, secondly, because of its framing not as an issue of remoteness but as one concerned with ‘the scope of the defendant’s duty of care’”.
- ❑ “[T]he relationship between the ‘scope of duty’ question and the traditional remoteness enquiry focused on foreseeability is obscure. ...[I]t is not at all obvious why both concepts are thought to be necessary”.

(Nolan & Plunkett, ‘Keeping Negligence Simple’)



5. Coincidental Harms

Chester v Afshar [2005] UKHL 41



Facts:

- ❑ C had a history of back pain, and her surgeon, D, advised her to have spinal surgery.
- ❑ D failed to disclose a 1% risk of serious nerve damage involved in an operation.
- ❑ The risk eventuated and harmed C.
- ❑ C admitted that, had she been informed, she would have sought advice and would have delayed having the operation, not have avoided it altogether.

Chester v Afshar [2005] UKHL 41



Held:

- ☐ D fully liable
- ☐ Emphasis on the importance of the duty on doctors to ensure patients can exercise choice over their own behaviour.
- ☐ Majority: Lords Steyn, Hope and Walker
- ☐ Minority: Lords Hoffmann and Bingham

Chester v Afshar [2005] UKHL 41



The problem:

- ❑ Because C would have had the operation later anyway, D's negligence made no difference to the risks for her.
- ❑ SAAMCO rule: "the law limits liability to those consequences which are attributable to that which made the act wrongful".
- ❑ The duty is to protect the patient from risks they were unwilling to accept – the damage is outside the scope of of that duty, because this is not a risk C was unwilling to accept.

Chester v Afshar [2005] UKHL 41



The majority view:

- Informed consent
- The patient's right to choose
- ❑ “[C’s] right of autonomy and dignity can and ought to be vindicated by a narrow and modest departure from traditional causation principles.” (Lord Steyn, [24])

Chester v Afshar [2005] UKHL 41



The majority view:

“To leave the patient who would find the decision difficult without a remedy, as the normal approach to causation would indicate, would render the duty useless in the cases where it may be needed most. This would discriminate against those who cannot honestly say that they would have declined the operation once and for all if they had been warned. I would find that result unacceptable.”

(Lord Hope, [87])

Chester v Afshar [2005] UKHL 41



The dissents:

“In my opinion this argument is about as logical as saying that if one had been told, on entering a casino, that the odds on the number 7 coming up at roulette were only 1 in 37, one would have gone away and come back next week or gone to a different casino. The question is whether one would have taken the opportunity to avoid or reduce the risk, not whether one would have changed the scenario in some irrelevant detail.” (Lord Hoffmann, [30])

Chester v Afshar [2005] UKHL 41



Criticism:

- ❑ In its rush to embrace the idea of informed consent and a patient's right to choose, the HoL undermined basic principles of causation, and unjustly compensated C.
- ❑ Clark and Nolan argue that if the Court was serious about patient autonomy, then it should have recognized autonomy violations as actionable damage on their own! (Clark & Nolan, 'A Critique of *Chester v Afshar*')
Chester v Afshar
- ❑ Ripe to be overruled?

Summary of legal causation

1. Was the type of harm reasonably foreseeable?
2. Was there an intervening act which was DIVU?
3. Was there a supervening natural event?
4. Was the damage suffered by C attributable to what made D's act wrongful?
5. Is there a special reason to hold D liable for damage outside the scope of their duty?